



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : زينب محمود شحاته داود
 المحافظة : القاهرة
 الحي / المدينة : الحسين
 رقم البطاقة : 24911121702049 رقم التليفون : 01063372415
 العمر (تاريخ الميلاد) : 12-11-1949 المهنة : بدون عمل
 العنوان : المحموديه
 تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
 الإخصائي الاجتماعي
 التوقيع: (.....)
 المقر بما فيه
 التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
 نتائج الأبحاث : (مرفق)
 التشخيص النهائي : سرطان بالثدي (مبادرة)
 خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
 التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة:
 الوظيفة:
 الوظيفة:
 التوقيع:
 التوقيع:
 التوقيع:
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

ختم
 شعار الجمهورية

مدير عام
 مديرية الشؤون الصحية

رئيس
 المجلس الطبي المتخصص

Name: زینب محمود شحاته داءد
National ID: 24911121702049



تقرير طبي

السيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين





مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



06/10/2026

MDT FORM

Name:	Zainab Mahmoud Shehata Dawood	MRN:	3498
Age:	77 Y	ID:	24911121702049
Menopausal Sta:	postm	Phone:	01063372415
NRS SCORE:	ps 1	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

R

L

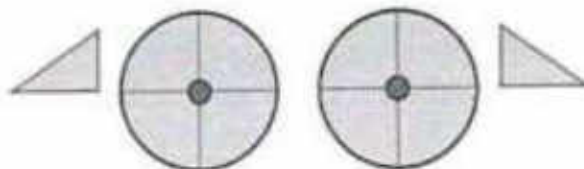
BX:
 ER: PR: HER2: KI67:

Sonomammography:Mets Workup:History:

postm rt breast cancer rt mmm on 27/7/2025 t2 n2 5/10 ldc 3 er8/8 pr5/8
 her23+ pt rec: 12 weeks taxol only 3 herceptin last 28/1 rec port femara from
 4/2026 no approval of herceptin from 1/2026 till now for femara for mdt (pt is
 high risk for phego.)

Recommendations:MDT Outcome & Plan

patient underwent upfront Rt MRM (7/2025) :pT2N2a received adjuvant Taxol
 with Herceptin and now on hormonal tit indicated to continue Herceptin for
 1year with AI.
 assessment with CT CAP



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: زبيب محمود شحاته داود		
MR. : 3498		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient underwent right mastectomy with right axillary node dissection on 27/7/2025. Pathology revealed a T2 N2 invasive ductal carcinoma, grade 5/10, estrogen receptor (ER) 8/8 positive, progesterone receptor (PR) 5/8 positive, HER2 2+ (borderline). The treatment plan included 12 weeks of taxol monotherapy followed by three doses of trastuzumab (herceptin) completed on 28/1. Because of lack of approval for trastuzumab from 1/2026 onward, the patient was recommended to start femara (letrozole) in April 2026, with a pending approval for anti-HER2 therapy (phasgo) due to high-risk disease. An echocardiogram was ordered to assess eligibility for anti-HER2 therapy once approval is obtained. The patient has completed 12 weeks of taxol. The patient is currently being managed with an aromatase inhibitor (AI) since April 2026. An echocardiogram was scheduled for today, and approval for anti-HER2 therapy remains pending. The patient has been on AI for 3 months. The patient continues on hormonal therapy with an aromatase inhibitor (AI), specifically femara, initiated today for a planned course of 3 months. The patient is currently receiving femara for a planned duration of 3 months.		
تحريراً في ,		
	التاريخ : 19-09-2026	الوقت :
nada mohamed elsayed		الطبيب المعالج :
يعتمد ,		



Histopathology report

Report Number 73

Female

التاريخ

26/05/2025 12:39

28/05/2025 14:11

تاريخ استلام العينة

تاريخ التقرير

شروع

الوقت

رقم العينة

15 Y

المريض

مركز صحة المرأة

مركز صحة المرأة

مركز صحة المرأة

Primary sample :

right Breastcore Bopsy

comment :

Gross examination :

Four cores of greyish white tissue . (T.I)

Microscopic examination

Section examined revealed fibrocollagenous tissue infiltrated by malignant growth displaying the morphology of infiltrating duct carcinoma

Referral Lab :

Comment :

Diagnosis

RIGHT BREAST CORE THE PICTURE IS COMPATIBLE WITH INFILTRATING DUCT CARCINOMA, GRADE II

Pathologist

Dr.Johan Aziz

Lab Director.

التوقيع



Immunohistochemistry report

Report Number m623

Female

المرأة

التاريخ

التاريخ

28/05/2026 09:11

تاريخ الميلاد

01/06/2025 10:30

تاريخ التقرير

رئيسة

76 Y

مركز صحة المرأة

مركز صحة المرأة

مركز صحة المرأة

مركز صحة المرأة

مركز صحة المرأة

مركز صحة المرأة

مركز صحة المرأة

مركز صحة المرأة

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER & PR : Harvey scoring system) - Her2neu : CAP scoring system)

Comment :

Technique :

sample received was treated by : - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody - CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody
Sample received was treated by : - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark QX / ULTRA
Sample received was treated by : - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER strong nuclear staining of 80 % of tumour cells
PR strong nuclear staining of 10 % of tumour cells
HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells
Ki67 positive nuclear staining of about 20 % of tumour cells

Referral lab :

Comment :

Conclusion :

ER POSITIVE (SCORE 8/8)
PR POSITIVE (SCORE 5/8)
HER2/NEU PROTEIN OVER EXPRESSION POSITIVE (SCORE 3 +)
#K67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 20 %
(CORE BIOPSY)

Pathologist:

التوقيع

Lab Director:

التوقيع

Patient Name : الأستاذة / زيات محمد شحاته
Patient No. : 1671-2025
Path No. : 2376/25
Sex : Female
Age : 67 Y
Referring Doc :

Request No. : 13008138
Request Date : 27/07/2025
Verification Date : 03/08/2025
Financial Category : جوازات روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.
Nature Specimen: Right modified radical mastectomy.

Gross Examination:

- * Right modified radical mastectomy specimen measured 23x17x5 cm, with covering skin ellipso measured 21x16 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed a mass 4.5x4x2 cm, located 2 cm from the deep margin.
- * Attached axillary fat 7x6x2 cm, entangling 10 lymph nodes, the largest measured 2x1x0.5 cm.

11 Blocks:

- 1:5- Mass., 6- N&A, 7- Deep margin., 8:11- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic Examination:

- * Histological Type: Invasive lobular carcinoma (pleomorphic subtype) with focal ductal component.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * 5 % High grade ductal carcinoma in situ (Solid pattern with comedo type necrosis & calcification).
- * Angiolinvasion: Present.
- * Perineural invasion: Absent.
- * Tumor infiltrating lymphocytes (TILs): 20%.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Lymph nodes: Positive metastatic deposits in five out of ten lymph nodes (5/10) with extra-nodal extension.

Verified by :
Dr. Heba Hasan

Printing Date : 12/08/2025 09:20:31

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - دور الأرض - مباني رقم (987) - دورشوش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / زينة محمود شحات
Patient No. : 1671-2025
Path No. : 2376/25
Sex : Female
Age : 67 Y
Referring Doc :

Request No. : 13008138
Request Date : 27/07/2025
Verification Date : 03/08/2025
Financial Category : جوفات روسي - فئة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Diagnosis:

- Right breast cancer, Right modified radical mastectomy:
Invasive lobular carcinoma (pleomorphic subtype) grade 3, with focal ductal component.
- 5% High grade ductal carcinoma in situ (Solid pattern with comedo type necrosis & microcalcification).
- Deep Margin: Free.
- Nipple & areola: Free.
- Lymph nodes: Positive metastatic deposits in five out of ten lymph nodes (5/10) with extra-nodal extension.
- Stage: pT2, pN2a.

Signature

Prof. Dr. Maha Guimef.
Dr. Heba El-Shafeey

Verified by :
Dr. Heba Hasan

Printing Date : 12/08/2025 09:20:31

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



جرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Name: Zelnab Mahmoud Shehata

R:

Date: 24/6/2025

Prof DR.

I. C.T. SCAN OF THE CHEST REVEALED:

- Clear both lungs showing no definite nodules masses or cavitary lesions.
- Clear both costophrenic recesses.
- Mild cardiomegally with no pericardial effusion
- No evidence of hilar/mediastinal lymphadenopathy

II. C.T. ABDOMEN AND PELVIS REVEALED:

- Mild hepatomegaly showing homogenous texture and regular smooth outlines. No definite focal lesions or HBRs dilatation
- Patent homogeneously enhancing main portal vein and its central branches
- Cholecystectomy.
- Average sized spleen showing no definite focal lesions.
- Good power of contrast excretion and concentration by both kidneys showing average size, preserved parenchymal thickness and smooth outlines
- Evidence of hysterectomy.
- Normal rest of the scanned pelvis-abdominal organs
- No retro-crural or retro-peritoneal lymph nodes are noted
- No ascites.

Dr. Ahmed Badr
SA

Prof. Dr. Maydy Bassiouni

33365450-37601208-37601207
33365450-37601208-37601207

Patient Name: Zainab Mahmoud Shehata
Dawood

Date : 26/07/2025

n : DUPLEX VIENS & ARTERIES LEFT
LOWER LIMB

Age : 75 Year(s)

Medical ID: 1671/2025

Pat. ID: 110659

**plex Examination of the arterial system of The both
lower limb revealed: -**

T SIDE:

Patent arterial system including common femoral, superficial femoral, popliteal, anterior and posterior tibial and dorsalis pedis arteries with normal intimal thickness.

Preserved arterial triphasic waveform configuration with average flow velocities of the examined arterial tree.

No evidence of arterial stenosis or occlusion seen.

SIDE:

Patent arterial system including common femoral, superficial femoral, popliteal, anterior and posterior tibial and dorsalis pedis arteries with normal intimal thickness.

Preserved arterial triphasic waveform configuration with average flow velocities of the examined arterial tree.

No evidence of arterial stenosis or occlusion seen.

ON: -

Normal arterial study.

Best Regards,
Dr. Rasha Elvashini

Patient ID: 12321
13.12.2025

Patient Name: Eng. A. / 21-05-2025 / 150kg / 170cm / 45y
BIRADS: 5
DDMMYY/05/25 03:25:12 AM / 03/05/2025

Women's Health Outreach Program

Name: Zelnab Mahmoud

Date: 25-5-2025

Bilateral Full field Digital Mammography

Clinical summary:

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibro glandular densities (ACR density c).
- An UOQ Area of distortion mounting to mass formations , with surrounding distortion.
- No suspicious clusters of micro calcifications.
- Ultrasound demonstrated normal parenchyma with about 9 o'clock extending from about 4 CMFN to about 1 CMFN ill defined lesion seen measuring about 4x4.6 cm with surrounding distortion , with minimal ductal vascularity
- No solid or cystic lesion
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- A Small rounded axillary Ln seen measuring about 9x8 mm.

Left breast

- Scattered fibro glandular densities (ACR density C).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated normal parenchyma
- No solid or cystic lesion
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Left breast (BIRADS 1): Normal

Right breast (BIRADS 5): An ill-defined hypoechoic suspicious looking lesion with intermediate right axillary LNs for histopathology correlation

Norhan osama, MD

Name : Zenab Mahmoud
Date : Monday, June 23, 2025

• Liver:

Average size liver of diffuse pathology (for correlation with liver function) normal shape and uniform echo pattern smooth surface. No focal lesion or dilated intrahepatic biliary radicles. Main portal vein is not dilated. CBD 10 mm

• Gall bladder

Surgically removed

• Spleen

Average size and homogenous texture no focal lesion splenic vein not dilated

• Pancreas:

concealed by gases

• Kidneys:

both of normal shape, average size and parenchymal thickness. No cysts, calculi or back pressure changes. ? grade 1 Pathological kidneys parenchyma for correlation with kidney function.

• Polyps:

Normal urinary bladder wall no stones or masses

Conclusion:

- liver of diffuse pathology (for correlation with liver function), mild dilated CBD
- ? grade 1 Pathological kidneys parenchyma for correlation with kidney function.
- Colonic gaseous distension

Signature
Dr Walced El Nabwey MD

٢٢١.١١.٨ - ٢٢١.١١.٧ : تاريخ

مركز جامعة الدول العربية - القاهرة

Name : Zainab Mahmoud Shehata Dawood
Branch : Dokki
Ref.Physician Prof : Gostaf Rosch Hospital

Date : 08-09-2025 09:26
Medical No. : 196947

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 76-year-old lady, with history of right breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



فيلية العامة للتعليمات والدراسات
مستشفى أحمد ماهر التعليمي
The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital

To whom it may concern

Mrs. / Mr. زينة محمد شهاب 76 years old

female patient presented with diagnosis

Rt. breast Ca.

She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She / He has received (3D/IMRT) radiotherapy to

Rt. CW and PCINs

At Dose : 46 Gy/15 Fr/3 w.

Additional (Boost) ; yes/ no. at dose Gy/ Fr/ w
(sequential / concomitant)

Started: 9/4/2024

Ended: 29/4/2024 With good tolerance .

Head of department
Dr. Henry William

د. هنري



الجمعية الشرعية

لتعاون العاملين بالكاتب والسنة المحمدية
مركز المغفرة الإسلامية للشهرة برقم ١٧٠٠ لسنة ٢٠٠٦
مركز المغفرة الإسلامية (معمل التحاليل)

Patient Name	الحاجة/ زينب محمود شحاته	Patient ID.	 1026090310187
Age / Sex	77 Years / Female	Request Date	03-Sep-2026 08:36 PM
Referred By	Herself	Lab ID	900260903090

BIOCHEMISTRY REPORT

Kidney Functions :

Reference Range :

▲ Serum Creatinine	:	1.0	mg/dL	Normal: 0.5 - 1.2 mg/dL
▲ Blood Urea	:	41	mg/dL	Female 15 - 50 mg/dL

Doctor's signature:

Thanks



ECHO REPORT

NAME: 3333333333
ID NUMBER: 1

DATE: 6/19/2026
SEX: 1 AGE: 1

EF	62%	55-75%	PWT	6.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVEDD	2.3	2.4cm	LA	3.5	2.4cm
FS	28%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function GI
SWMA Normal
LA and Ao Root Dimensions Normal
Mitral Valve Trace MR
Aortic Valve Normal
Tricuspid Valve Mild TR
Pulmonary Valve Trace PASP = 35 mmHg
Right Side and Systolic Function Normal
Intracardiac Masses and Effusion Normal

DR

26/1/26



Immunohistochemistry report

Report Number: m623

Female	الجنس	زيتب محمود شحاته	اسم المريض
التاريخ :	الفرم	76 Y	السن
28/05/2025 09:11	تاريخ استلام العينة		تاريخ اخذ العينة :
01/06/2025 10:30	تاريخ التقرير		طالب التحليل :
			مركز صحة المرأة
			مركز صحة المرأة
			عناية

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by :- CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody - CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by :- VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / ULTRA.

Sample recieved was treated by :- CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER: strong nuclear staining of 80 % of tumour cells

PR: strong nuclear staining of 10 % of tumour cells

HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells.

Ki67 :positive nuclear staining of about 20 % of tumour cells

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 8/8)

PR: POSITIVE (SCORE 5/8)

HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3 +)

#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 20 %
(CORE BIOPSY)

Pathologist

دكتور علاء ماهر

Lab Director:

دكتور علاء ماهر

وزارة الصحة
مركز صحة المرأة
بتم 30



جمعية مسجد د. مصطفى محمود
Dr. Mostafa Mahmoud Mosque
مستشفى الاعلام
وحدة الأشعة العلاجية وأشعة الثدي

Mammography & X - Ray unit

Name ZINAB MAHMUD SHHATA	Date : 7/9/2026
Age : 77 TE : 01063372415	Exam : MAMMO

- Clinical presentation: right mastectomy.

MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND FOR LEFT BREAST REVEALED:

- Clear right sided operative bed
- Dense mammary fibro glandular architecture.
- left breast mottling parenchyma
- left breast 3 o'clock well defined benign looking hypo echoic nodules measured 8x7 mm and 6x6 mm
- No axillary lymphadenopathy
- No irregular mass lesion identified
- Normal skin thickness and contour of the breast
- BIRADS 3 for follow up

Best regards,

Dr. Ennan El Tawel.MD

(Dear Prof doctor For Further medical questions please do not hesitate to contact me
on what's up number +20 10 60442675)

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BIRADS 0 : Needs further investigations.
- BIRADS 1: Normal findings.
- BIRADS 2: Benign.
- BIRADS 3: Probably benign.
- BIRADS 4: A: low suspicious of malignancy
B: moderate suspicious of malignancy
C: Highly suspicious of malignancy
- BIRADS 5: Malignant.
- BIRADS 6: Proven pathologically malignant

MDT FORM 8/7/2025 G

Name	زينب محرم حجاز			MRN	1111/100		
Age	47			ID	03911111111111111111		
Menopausal status	Post			Phone	11.7777777777777777		
NRS SCORE				Marital status			
BMI				Family history			
CO morbidity	Heart, Free Hypertension, Fibroid.			REASON OF MDT	New		
Clinical Data							
BX:	IDC G3			R	L		
ER:	PR:	HER 2:	Ki67:				
8/8	5/8	3+	20%				
Sonomammography:							
Mets Workup:				TNM₂ Clinically: R- But 9:00 5x 4.5 cm mass.			
Recommendations:				For Possibility of neoadj HT then BCS if there is contr indication for neoadj HT upfront surgery can be done. Ominifex 2/7/2025			
MDT OUTCOME & PLAN							
for upfront R+. BCS + SLNB							
SIGNATURE							
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric		
	Dr. Hany 8/7/2025						

Patient Name	: الأستاذة / زينب محمود شحاته	Request No.	: 13008138
Patient No.	: 1671-2025	Request Date	: 27/07/2025
Path No.	: 2376/25	Verification Date	: 03/08/2025
Sex	: Female	Financial Category	: جوفات روسي - نفقة الزوجة
Age	: 67 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature Specimen: Right modified radical mastectomy.

Gross Examination:

- * Right modified radical mastectomy specimen measured 23x17x5 cm, with covering skin ellipse measured 21x16 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a mass 4.5x4x2 cm, located 2 cm from the deep margin.
- * Attached axillary fat 7x6x2 cm, entangling 10 lymph nodes, the largest measured 2x1x0.5 cm.

11 Blocks:

- 1:5- Mass., 6- N&A., 7- Deep margin., 8:11- ALNs.
- Gross was done by: Dr. Heba El-Shafeey.

Microscopic Examination:

- * Histological Type: Invasive lobular carcinoma (pleomorphic subtype) with focal ductal component.
- * Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * 5 % High grade ductal carcinoma in situ (Solid pattern with comedo type necrosis & calcification).
- * Angiolinvasion: Present.
- * Perineural invasion: Absent.
- * Tumor infiltrating lymphocytes (TILs): 20%.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Lymph nodes: Positive metastatic deposits in five out of ten lymph nodes (5/10) with extra-nodal extension.

Verified by :

Dr.Heba Hasan



Patient Name : الأستة / زينة محمود شحات
Patient No. : 1671-2025
Path No. : 2376/25
Sex : Female
Age : 67 Y
Reffering Doc :

Request No. : 13008138
Request Date : 27/07/2025
Verification Date : 03/08/2025
Financial Category : فئة الزرلة - جزيئات روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Diagnosis:

Right breast cancer, Right modified radical mastectomy.

Invasive lobular carcinoma (pleomorphic subtype) grade 3, with focal ductal component.
- 5% High grade ductal carcinoma in situ (Solid patten with comedo type necrosis & microcalcification).

- Deep Margin: Free.

- Nipple & areola: Free.

- Lymph nodes: Positive metastatic deposits in five out of ten lymph nodes (5/10) with extra-nodal extension.

- Stage: pT2, pN2a.

Signature

Prof. Dr. Maha Guimei.

Dr. Heba El-Shafeey

Verified by :

Dr.Heba Hasan



تَحِيَّةٌ طَيِّبَةٌ وَبَعْدُ

Page 1 of 1

Mammography & X - Ray unit

Name ZINAB MAHMUD SHHATA	Date : 7/9/2026
Age : 77 TE : 01063372415	Exam : MAMMO

- Clinical presentation: right mastectomy.

MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND FOR LEFT BREAST REVEALED:

- Clear right sided operative bed
- Dense mammary fibro glandular architecture.
- left breast mottling parenchyma
- left breast 3 o'clock well defined benign looking hypo echoic nodules measured 8x7 mm and 6x6 mm
- No axillary lymphadenopathy
- No irregular mass lesion identified
- Normal skin thickness and contour of the breast
- BIRADS 3 for follow up

Best regards,

Dr. Emran El Taweel MD

(Dear Prof doctor For Further medical questions please do not hesitate to contact me on what's up number +20 10 60442675)

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BIRADS 0 : Needs further investigations.
- BIRADS 1: Normal findings.
- BIRADS 2: Benign.
- BIRADS 3: Probably benign.
- BIRADS 4: A: low suspicious of malignancy.
B: moderate suspicious of malignancy
C: Highly suspicious of malignancy
- BIRADS 5: Malignant.
- BIRADS 6: Proven pathologically malignant

Date:- 5/9/2026

Prof DR.

- CHEST:-

- ## II- ABDOMEN:-

- Opinion:

- Dr. Fatma Naem**

Prof. Dr. Tamer Ahmed Kamal

٣٣٣٦٠٤٥٠ - ٣٧٦ . ٤٢٠٨ - ٣٧٦ . ٤٢٠٧٤ العربية النوى جامعة الدول - مؤيد محمد محمود - مؤيد الانشطة - مؤيد